

Policy on End-of-Life Care

«Hospital Name»

Document control

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Issue No.	01	Review due	One year from implementation
Date created	_____	Date of implementation	_____
Prepared by	Palliative / End-of-Life Care Lead Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

End-of-life care is provided compassionately and consistently, respecting the patient and family's values and wishes, with DNR, DNI, and Allow Natural Death decisions made only within the applicable legal and judicial framework.

1. Purpose

This policy explains how «Hospital Name» provides end-of-life care under written guidance, uses a multi-professional team, ensures decisions like DNR, DNI, and Allow Natural Death follow the law, identifies and responds to the unique needs of dying patients and their families, and provides symptomatic treatment and pain relief.

This policy does not cover general informed consent procedures in detail — that is covered in other hospital policies.

2. Scope

This policy applies to all clinical and support staff providing care to patients at the end of life at «Hospital Name».

3. Policy standards

«Hospital Name» provides end-of-life care under written guidance based on national and international good practice and the law of the land, using a multi-professional approach where possible. Decisions like DNR, DNI, and Allow Natural Death are made only within the

applicable statutory and judicial framework. The unique psychological, emotional, cultural, religious, and socio-cultural needs of the patient and family are identified and respected, staff are trained accordingly, and symptomatic treatment and pain relief are provided with the patient and family involved throughout.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not provide end-of-life care without written guidance covering pain, rehabilitative, and palliative care aligned with the patient and family's wishes; sensitively addressing autopsy and organ donation; respecting the patient's values, religion, and culture; involving the patient and family in every aspect of care; and responding to their psychological, emotional, spiritual, and cultural concerns where possible.
- Do not make a DNR, DNI, Allow Natural Death, or similar decision except as the applicable statutory laws and judicial guidelines require.
- Do not fail to identify and respond to the patient and family's psychological, emotional, cultural, religious, and socio-cultural needs, and do not skip training staff on these unique needs.
- Do not exclude the patient or family from decisions about symptomatic treatment and comfort care.

5. What we do

5.1 Provide end-of-life care under written guidance

Written guidance for end-of-life care is developed based on national and international good practice and in line with the law of the land. It covers five areas: giving appropriate pain, rehabilitative, and palliative care in line with the patient and family's wishes; sensitively addressing issues like autopsy and organ donation; respecting the patient's values, religion, and cultural preferences; involving the patient and family in every aspect of care; and responding to their psychological, emotional, spiritual, and cultural concerns, where possible. This guidance also incorporates the legal framework for DNR, DNI, and similar decisions described in Section 5.3, and is prepared with the multi-professional approach in Section 5.2 in mind.

5.2 Use a multi-professional approach

End-of-life care is provided using a multi-professional approach — the team could include a doctor, nurse, clinical psychologist, and rehabilitation expert, among others. It's preferable for someone trained in palliative care to lead the team.

5.3 Follow the legal framework for end-of-life decisions

Decisions like Do Not Resuscitate, Do Not Intubate, and Allow Natural Death are made only as the applicable statutory laws and the guidelines framed by the legal system require — for

example, the ICMR Do Not Resuscitate Guideline and the Supreme Court's 2023 directives on Advance Directives and Living Wills. «Hospital Name» is responsible for verifying the current version of these and any other applicable statutory or judicial requirements, including any State-specific rules.

5.4 Identify and respond to unique needs

The psychological, emotional, cultural, religious, and socio-cultural needs of the patient and family are identified, addressed, and respected. Staff are trained on the unique needs of patients and families at the end of life.

5.5 Provide symptomatic treatment and involve the patient and family

The emphasis is on providing symptomatic treatment and preventing complications as far as possible. The patient and/or family are involved in every such decision, and their choices are respected to the extent possible and as the law allows. Where appropriate, measures are taken to relieve pain.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced.
Palliative / End-of-Life Care Lead	Owns day-to-day implementation; maintains written guidance covering all five content areas; convenes and coordinates the multi-professional team; ensures DNR/DNI/AND decisions are made only within the applicable statutory and judicial framework; ensures staff training on unique needs of end-of-life patients and families.
Treating doctors	Provide symptomatic treatment and pain relief; involve the patient and family in all relevant decisions; make DNR/DNI/AND decisions only within the legal framework; document decisions in the medical record.
Nursing and support staff	Identify and respond to the psychological, emotional, cultural, religious, and socio-cultural needs of the patient and family; involve the patient and family in care decisions; follow written guidance.
Multi-professional team members	Participate in collaborative end-of-life care planning and delivery as appropriate to their role (e.g., clinical psychologist, rehabilitation expert).
Quality Coordinator	Audits this policy quarterly; holds training acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
Written guidance	Written guidance for end-of-life care in place, based on national and international good practice and law of the land; covers all five content areas; incorporates the DNR/DNI/AND legal framework; reviewed regularly
Multi-professional approach	Multi-professional team involved in end-of-life care; team composition documented
DNR / DNI / AND decisions	Every DNR, DNI, Allow Natural Death, or similar decision made only as the applicable statutory laws and judicial guidelines require; documented with basis for the decision; current applicable instruments verified
Unique needs — identified and responded to	Psychological, emotional, cultural, religious, and socio-cultural needs of patient and family identified, addressed, and respected; all five dimensions evidenced
Staff training on unique needs	Staff trained on unique needs of end-of-life patients and families — separate mandatory obligation; training records on file
Symptomatic treatment and family involvement	Symptomatic treatment and pain relief provided; patient and/or family involved in all such decisions; choices respected within legal limits; documented

8. Training and staff acknowledgement

All clinical and support staff shall be familiar with the requirements of this policy, including: the five mandatory written-guidance content areas (pain/rehabilitative/palliative care aligned with patient and family wishes; sensitively addressing autopsy and organ donation; respecting values/religion/culture; involving patient and family; responding to psychological/emotional/spiritual/cultural concerns where possible); the absolute framing that DNR/DNI/AND decisions are made only as applicable statutory laws and judicial guidelines require; the five need dimensions in Section 5.4 (psychological, emotional, cultural, religious, and socio-cultural — all five must be identified, addressed, and respected — both verbs mandatory); and staff training on end-of-life unique needs as a separate mandatory obligation.

I have read the Policy on End-of-Life Care of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
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_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to all clinical and support staff providing care to patients at the end of life at «Hospital Name».

10. Abbreviations

Abbreviation	Meaning
AND	Allow Natural Death
COP	Care of Patients (NABH Hospitals chapter)
DNI	Do Not Intubate
DNR	Do Not Resuscitate
ICMR	Indian Council of Medical Research
NABH	National Accreditation Board for Hospitals and Healthcare Providers

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.20.a	Commitment*	Section 5.1 addresses written guidance based on national and international good practice and law of the land, covering five mandatory content areas: (1) appropriate pain, rehabilitative, and palliative care aligned with patient and family wishes; (2) sensitively addressing autopsy and organ donation; (3) respecting values, religion, and cultural preferences; (4) involving patient and family in every aspect of care; (5) responding to psychological, emotional, spiritual, and cultural concerns where possible. The guidance incorporates

		the DNR/DNI/AND legal framework from COP.20.c.
COP.20.b	Achievement	Section 5.2 addresses a multi-professional approach to end-of-life care. Team composition is illustrative (doctor, nurse, clinical psychologist, rehabilitation expert). Someone trained in palliative care leading the team is preferable — advisory. Achievement level.
COP.20.c	Commitment	Section 5.3 addresses the absolute requirement that DNR, DNI, Allow Natural Death, and similar decisions are made only as the applicable statutory laws and guidelines framed by the legal system require. Named instruments are illustrative examples: ICMR DNR Guideline; Supreme Court 2023 directives on Advance Directives and Living Wills. The organisation is responsible for verifying the current version and any State-specific requirements. The word 'only' is absolute — no discretionary departure from the legal framework is permitted.
COP.20.d	Commitment	Section 5.4 addresses two distinct mandatory obligations: (1) identifying, addressing, and respecting the patient and family's unique needs across all five dimensions — psychological, emotional, cultural, religious, and socio-cultural (all five named; both 'addressed' and 'respected' are mandatory verbs); and (2) staff training on the unique

		needs of end-of-life patients and families — a separate mandatory obligation with its own training records requirement.
COP.20.e	Commitment	Section 5.5 addresses symptomatic treatment and complication prevention as the emphasis; patient and/or family involved in every such decision; their choices respected to the extent possible and as the law allows; pain alleviation measures taken where appropriate.

12. Required Records/Evidence Checklist

Written guidance (COP.20.a)

- Written end-of-life care guidance covering all five content areas, based on national and international good practice and law of the land, with the DNR/DNI/AND legal framework incorporated.

Multi-professional approach (COP.20.b)

- Records of multi-professional team involvement in end-of-life care cases — team composition documented.

DNR / DNI / AND decisions (COP.20.c)

- Documentation of every DNR, DNI, Allow Natural Death, or similar decision — confirming it was made within the applicable statutory and judicial framework; legal basis stated.
- Evidence that applicable instruments (ICMR DNR Guideline; Supreme Court 2023 directives; any State-specific requirements) are current versions verified by the organisation.

Unique needs — addressed and respected (COP.20.d)

- Records showing identification, addressing, and respect of all five need dimensions for each patient and family: psychological, emotional, cultural, religious, and socio-cultural.

Staff training on unique needs (COP.20.d)

- Staff training records on the unique needs of end-of-life patients and families — separate from general EOL care training records.

Symptomatic treatment and family involvement (COP.20.e)

- Records of symptomatic treatment and pain relief provided.
- Records confirming patient and/or family involvement in symptomatic treatment decisions.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.20.
2. Guidebook interpretation supplied for COP.20.a through COP.20.e.
3. Indian Council of Medical Research. ICMR Consensus Guidelines on 'Do Not Attempt Resuscitation' (current version — organisation to verify).
4. Supreme Court of India. Directives on Advance Directives and Living Wills, 2023 (current version — organisation to verify).

Disclaimer

This policy reorganises the supplied COP.20 objective elements and approved plain-language content into policy format. No formal stop-work section is included — COP.20 is not in the accreditation stop-work proposals. The five mandatory written-guidance content areas (COP.20.a) are preserved exactly and listed in Section 5.1, the Non-negotiable rules, the quality monitoring table, the traceability table, and the training section. The absolute framing for DNR/DNI/AND decisions — 'only as the applicable statutory laws and judicial guidelines require' — is preserved verbatim in the Statement of intent, Section 3, Section 5.3, the Non-negotiable rules, the quality monitoring table, and the traceability table; 'only' is not qualified or softened. All five need dimensions (psychological, emotional, cultural, religious, socio-cultural) are stated explicitly in Section 5.4, the Non-negotiable rules, Section 3, the quality monitoring table, the traceability table, and the training section. Both mandatory verbs — 'addressed' and 'respected' — are preserved in Section 5.4 and the traceability table. Staff training on unique needs is stated as a separate mandatory obligation in Section 5.4, the Non-negotiable rules, the quality monitoring table (separate row), the traceability table, and the evidence checklist (separate sub-section with its own record requirement).